Antisocial Personality Disorder (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Antisocial personality disorder (ASPD) is a pervasive pattern of disregard for and violation of the rights of others, occurring since age 15, in a person at least 18 years old who showed **conduct disorder before age 15** (American Psychiatric Association [APA], 2022). It belongs to **Cluster B** (dramatic or erratic).

How common: in a large U.S. national survey, 3.63% of adults met criteria (Grant et al., 2004). Rates are far higher in prisons and addiction treatment settings.

Course: ASPD usually begins around age 8 as conduct problems and is lifelong for most people, but it **tends to improve with age**. Earlier onset predicts a worse course; marriage, steady employment, and better socialization are linked to improvement. ASPD often co-occurs with substance use, other mental disorders, and medical illness, and rates of natural and violent death (suicide, homicide, accidents) are high (Black, 2015).

Counselor framing: many clients with ASPD come to counseling under pressure from courts, employers, or spouses. Expect manipulation and testing, but also recognize real suffering, losses, and, in some, a genuine desire to change.

2. Core symptoms to watch for

DSM-5-TR criteria, paraphrased (APA, 2022); use the manual itself for formal diagnosis. Since age 15, **3 or more** of:

- Repeated **unlawful behavior** that could lead to arrest.
- **Deceitfulness:** repeated lying, use of aliases, or conning others for profit or pleasure.
- **Impulsivity** or failure to plan ahead.
- **Irritability and aggressiveness,** with repeated fights or assaults.
- **Reckless disregard** for the safety of self or others.
- **Consistent irresponsibility,** such as failing to hold a job or pay debts.
- **Lack of remorse,** shown by indifference to or rationalizing having hurt, mistreated, or stolen from others.

Also required: age at least 18; evidence of conduct disorder before age 15; and the behavior does not occur only during schizophrenia or bipolar disorder (APA, 2022).

Psychopathy is narrower. Not everyone with ASPD shows the callous, unemotional style of psychopathy. In youth, callous-unemotional traits mark a subgroup with more severe and persistent antisocial behavior that responds less well to standard treatment (Frick et al., 2014).

When counselors must refer (general clinical consensus): assess risk of violence to others, including partners and children, at every stage. Refer for psychiatric and addiction evaluation, follow duty-to-warn and mandated-reporting laws, and do not treat ASPD alone. Ask about suicide too (Black, 2015).

3. What works: treatment and the evidence

Bottom line for class: the evidence is weak. No treatment has clearly been shown to change antisocial behavior in adults with ASPD. Treating co-occurring substance use and building skills and structure are the most practical goals.

Evidence overview (honest effect sizes):

- **Cochrane review:** across 19 trials (data from 10 trials, 605 participants), only three approaches (contingency management added to standard maintenance treatment, schema therapy, and dialectical behavior therapy) showed benefit over control on a key outcome, and there was **no compelling evidence** of change in antisocial behavior. Overall certainty was low or very low (Gibbon et al., 2020).
- **CBT:** in an exploratory trial of 52 men with ASPD and recent aggression, adding CBT to usual care did not clearly reduce aggression at 12 months, though trends favored CBT for drinking, social functioning, and beliefs about others (Davidson et al., 2009).

- **Schema therapy** (which targets long-standing core beliefs) has shown benefit across several personality disorders (Bamelis et al., 2014), and was one of the few approaches with benefit in the Cochrane review (Gibbon et al., 2020).
- **Medication (referral notes):** no medication is approved for ASPD itself. Medication may help co-occurring substance use, ADHD, depression, or aggression and is managed by a physician.

Cognitive-behavioral work with ASPD: how it is used (Davidson et al., 2009; Gibbon et al., 2020):

- **Structure and accountability.** Clear rules, consistent consequences, and coordination with probation or treatment programs.
- **Identify and challenge core beliefs:** "Rules are for other people," "If I don't take it, someone else will," "People are weak and deserve to be used," "If I've been wronged, I have the right to get even."
- **Appeal to self-interest first,** then build empathy: show how antisocial choices cost the person freedom, relationships, and goals, and practice problem solving and anger control.

4. Religion, spirituality, and Christian integration: what the evidence shows

Religion and crime. Across 60 studies, religion had a moderate deterrent effect on criminal behavior (Baier & Wright, 2001).

Faith-based prison programs. In an 8-year follow-up of Prison Fellowship participants, there was no overall difference in time to rearrest, but inmates most active in Bible studies were less likely to be rearrested at 2 and 3 years after release (Johnson, 2004). In Minnesota, 732 released prisoners in the InnerChange Freedom Initiative, a faith-based program with cognitive-behavioral content and mentoring inside and outside prison, had significantly lower rearrest, reconviction, and reincarceration (Duwe & King, 2013). These studies included offenders generally, not only people diagnosed with ASPD.

Broader evidence. Across 97 outcome studies, therapy adapted to clients' religious beliefs worked about as well as standard therapy for distress and added spiritual benefits (Captari et al., 2018). We found **no trial of Christian-integrated treatment for ASPD** (literature search, November 1, 2026).

Honest summary for teaching:

1. No treatment reliably changes ASPD. Structured, skills-based care and treatment of addiction are the realistic foundation. Faith integration supports them; it does not replace them.
2. Faith-based programs with strong mentoring and evidence-based content show encouraging results for offenders, though not specifically for ASPD.
3. Be alert for religious language used to manipulate ("I'm saved now, so the past doesn't matter"). Look for changed behavior over time.

5. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within structured, cognitive-behavioral care. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Listen for entitlement and self-justification, including spiritual versions: "God forgives, so it doesn't matter," "Everybody sins, so why should I be judged?", "Those people had it coming" (Davidson et al., 2009).
- **Challenge (clinical).** Hold the person to specific behaviors and their consequences. Distinguish words of repentance from patterns of changed behavior, restitution, and honesty over months (Gibbon et al., 2020).
- **Replace (pastoral).** Offer Scripture that joins grace with responsibility:
    - "He who conceals his sins will not prosper, but whoever confesses and renounces them will find mercy." (Proverbs 28:13). Confession and change belong together.
    - "But Zacchaeus stood up and said to the Lord, 'Look, Lord, half of my possessions I give to the poor, and if I have cheated anyone, I will repay it fourfold.'" (Luke 19:8). Restitution as fruit of real repentance.
    - "Do nothing out of selfish ambition or empty pride, but in humility consider others more important than yourselves. Each of you should look not only to your own interests, but also to the interests of others." (Philippians 2:3–4)
    - "I will give you a new heart and put a new spirit within you; I will remove your heart of stone and give you a heart of flesh." (Ezekiel 36:26). Hope for the hardened heart.
- **Faith community (clinical).** Mature, well-boundaried mentors and accountability groups can help, as in the faith-based programs above (Duwe & King, 2013). Protect vulnerable members, finances, and children with clear safeguards.
- **Ministers:** welcome the person while keeping firm boundaries, never ignore threats or exploitation, and work with probation officers and clinicians.

6. Quick classroom takeaways

- ASPD means disregard for others' rights since 15 (3 of 7 criteria), age 18 or older, with conduct disorder before 15. About 3.6% of U.S. adults; it often eases with age. Assess violence and suicide risk.
- No treatment reliably changes ASPD; treat addiction, build structure and skills, and use schema or cognitive approaches.
- Pair grace with accountability: replace "rules are for other people" with confession, restitution, and a new heart, proven over time.

Sources note

All 11 references were checked against Crossref, OpenAlex, and PubMed/Europe PMC records (Nov 1, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record.

References

American Psychiatric Association. (2022). *Diagnostic and statistical manual of mental disorders (5th ed., text rev.)*. https://doi.org/10.1176/appi.books.9780890425787

Baier, C. J., & Wright, B. R. E. (2001). "If you love me, keep my commandments": A meta-analysis of the effect of religion on crime. *Journal of Research in Crime and Delinquency*, *38*(1), 3–21. https://doi.org/10.1177/0022427801038001001

Bamelis, L. L. M., Evers, S. M. A. A., Spinhoven, P., & Arntz, A. (2014). Results of a multicenter randomized controlled trial of the clinical effectiveness of schema therapy for personality disorders. *American Journal of Psychiatry*, *171*(3), 305–322. https://doi.org/10.1176/appi.ajp.2013.12040518

Black, D. W. (2015). The natural history of antisocial personality disorder. *The Canadian Journal of Psychiatry*, *60*(7), 309–314. https://doi.org/10.1177/070674371506000703

Captari, L. E., Hook, J. N., Hoyt, W. T., Davis, D. E., McElroy-Heltzel, S. E., & Worthington, E. L. (2018). Integrating clients' religion and spirituality within psychotherapy: A comprehensive meta-analysis. *Journal of Clinical Psychology*, *74*(11), 1938–1951. https://doi.org/10.1002/jclp.22681

Davidson, K. M., Tyrer, P. J., Tata, P. R., Cooke, D. J., Gumley, A. I., Ford, I., Walker, A., Bezlyak, V., Seivewright, H., Robertson, H., & Crawford, M. J. (2009). Cognitive behaviour therapy for violent men with antisocial personality disorder in the community: An exploratory randomized controlled trial. *Psychological Medicine*, *39*(4), 569–577. https://doi.org/10.1017/S0033291708004066

Duwe, G., & King, M. (2013). Can faith-based correctional programs work? An outcome evaluation of the InnerChange Freedom Initiative in Minnesota. *International Journal of Offender Therapy and Comparative Criminology*, *57*(7), 813–841. https://doi.org/10.1177/0306624X12439397

Frick, P. J., Ray, J. V., Thornton, L. C., & Kahn, R. E. (2014). Can callous-unemotional traits enhance the understanding, diagnosis, and treatment of serious conduct problems in children and adolescents? A comprehensive review. *Psychological Bulletin*, *140*(1), 1–57. https://doi.org/10.1037/a0033076

Gibbon, S., Khalifa, N. R., Cheung, N. H.-Y., Völlm, B. A., & McCarthy, L. (2020). Psychological interventions for antisocial personality disorder. *Cochrane Database of Systematic Reviews*, *2020*(9), CD007668. https://doi.org/10.1002/14651858.CD007668.pub3

Grant, B. F., Hasin, D. S., Stinson, F. S., Dawson, D. A., Chou, S. P., Ruan, W. J., & Pickering, R. P. (2004). Prevalence, correlates, and disability of personality disorders in the United States: Results from the National Epidemiologic Survey on Alcohol and Related Conditions. *The Journal of Clinical Psychiatry*, *65*(7), 948–958. https://doi.org/10.4088/JCP.v65n0711

Johnson, B. R. (2004). Religious programs and recidivism among former inmates in Prison Fellowship programs: A long-term follow-up study. *Justice Quarterly*, *21*(2), 329–354. https://doi.org/10.1080/07418820400095831

Prepared for Clinical Companion (Brian Campbell). Teaching overview only. Clinically review before use. Not a substitute for the DSM-5-TR or clinical judgment. All 11 references and DOIs verified via Crossref, OpenAlex, and PubMed/Europe PMC records (Sunday, Nov 1, 2026). Effect sizes as reported in the cited sources.